

22AVCV00223 22AVCV00223

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Case Number: 22AVCV00223 Hearing Date: September 22, 2026 Dept: A14

SUPERIOR COURT OF THE STATE OF CALIFORNIA

COUNTY OF LOS ANGELES – NORTH DISTRICT

ANA SALAZAR,

Plaintiff,

v.

CVS HEALTH; CVS PHARMACY; ANTINO RIVAS; and DOES 1 through 100,

Defendants.

Case Number 22AVCV00223

[TENTATIVE]

RULING ON MOTION

Date of Hearing: September 22, 2026

Dept. A-14

Judge William H. Forman

I. Background

This is a medical malpractice case. Defendant Antino Rivas ("Rivas") moves for summary judgment.

The operative complaint is a Second Amended Complaint ("SAC") filed on October 21, 2025. The SAC asserts causes of action for (1) Professional Medical Negligence (Medical

Malpractice); (2) Willful Misconduct; (3) Gross Negligence; (4) Fraudulent Concealment:

(5) Lack of Informed Consent; (6) Medical Battery; (7) Breach of Fiduciary Duty; and (8)

Negligent Hiring, Training, Supervision, and Retention.

The allegations of the SAC concern an alleged injury that Plaintiff suffered on April 7, 2021, when she went to a CVS in Lancaster, California to receive a COVID-19 vaccination

shot. Defendant Ifejika, an employee of CVS, gave Plaintiff an initial injection that caused

her to bleed at the injection site. (SAC, ¶ 30.) Plaintiff alleges that Ifejika "had injected a used needle" into her arm. (SAC, ¶ 33.) Plaintiff at times refers to this needle as "contaminated" or "used." Later in the SAC, she explains what she means by this: "The needle was used contaminated in that it was unwrapped, lacking any protected covering or

encasing, normally used to prevent accidental punctures. ... The used contaminated needle

lacked any needle cap or needle shield on or around its piercing bevel or point." (SAC, ¶ 49(b) and (c).) While previous versions of the complaint alleged that the needle was contaminated with a foreign substance, the SAC appears to have abandoned that allegation.

Plaintiff alleges that Ifejika the produced a second needle and injected Plaintiff with the contents of that needle. (SAC, ¶¶ 35-38.) Ifejika allegedly did not tell Plaintiff what was in that needle, and the SAC proceeds to allege that Ifejika surreptitiously disposed of the first needle. (Id.) Plaintiff alleges that she became sick and suffered emotional distress as a result of the shots she received at CVS when she went in to get her COVID-19 vaccination. (SAC, ¶¶ 43-44.)

On November 24, 2025, CVS, CVS Health, Rivas, and Ifejika each answered the complaint.

On April 1, 2026, Defendants each filed a motion for summary judgment.

On August 18, 2026, the Court granted CVS and CVS Health's motion for summary judgment.

On September 2, 2026, Plaintiff filed her opposition to Rivas' motion for summary judgment.

On September 11, 2026, Rivas filed his Reply.

II. Requests for Judicial Notice

Rivas' RJN – Rivas requests that the Court take judicial notice of the the March 10, 2020 Declaration of United States Health and Human Services Secretary Alex Azar invoking the Public Readiness and Emergency Preparedness Act for the COVID-19 pandemic effective February 4, 2020 (PREP Act).

Defendant's request is GRANTED. (Evid. Code, §§ 452 subd. (a), (c), (h), 453.)

Plaintiff's RJN – Plaintiff requests that the Court take judicial notice of (1) the October 7, 2025 Order After Hearing, (2) the May 14, 2024 Ruling, (3) Plaintiff's SAC, (4) California Business and Professions Code, Section 4113.1, (5) 85 Fed. Reg. 15,198, 15,202 (March 17, 2020), and (6) Coleman v. Sharp Memorial Hosp., No. 37-2023-00033307-CU-PO-CTL, 2024 Cal. Super. LEXIS 10893 (Cal. Sup. Ct. March 29, 2024).

Plaintiff's request is GRANTED. (Evid. Code, §§ 452, subd. (a), (c), 453.) The Court notes that judicial notice of filings within the instant action is unnecessary, and further notes that Coleman is not binding or precedential on this Court.

III. Evidentiary Objections

Rivas' Evidentiary Objections – Rivas objects to portions of Plaintiff's declaration and the Declaration of Ruwanthi Campo, M.D.

Defendant's objections to Plaintiff's Declaration are SUSTAINED as to No. 11 (D'Amico).

Defendant's objections to Plaintiff's Declaration are OVERRULED as to Nos. 1-10, 12.

Defendant's objections to the Declaration of Ruwanthi Campo, M.D. are SUSTAINED as to Nos. 14, 15, 17, 18.

Defendant's objection to the Declaration of Ruwanthi Campo, M.D. are OVERRULED as to Nos. 13, 16.

Plaintiff's Evidentiary Objections – Plaintiff objects to portions of the Declaration of Kim Moore.

Plaintiff's objections are SUSTAINED as to the following as improper legal opinion:

1. The administration of COVID-19 vaccines in April 2021 constituted the administration of a covered countermeasure under the Public Readiness and Emergency Preparedness ("PREP") Act. See Paragraph 8(b)
2. All actions taken by covered persons including but not limited to pharmacy, pharmacists, its agents, and employees in connection with the preparation and administration of COVID-19 vaccines, including decisions made during the injection process to ensure that a patient received the vaccine, fall within the scope of covered activities under the PREP Act. At all relevant times, Defendants, including CVS and its pharmacists, were "covered persons" under the PREP Act. See Paragraph 8(c)

3. Moreover, Defendants actions with respect to the April 7, 2021 preparation of and administration of Plaintiff's COVID-19 vaccine fall within the scope of covered activities, and as such are immune from liability, under the PREP Act. See Paragraph 8(p)

Plaintiff's remaining objections to the Declaration of Kim Moore are OVERRULED.

Plaintiff also objects to portions of the Declaration of Mark Needham, MD.

Plaintiff's objection is SUSTAINED as to the following as improper non-expert opinion:

1. Further, Defendant Antino Rivas was not involved in the April 7, 2021, vaccination at issue in this case and as such, to a reasonable degree of medical probability, nothing that Defendant Rivas did or failed to do caused or contributed to Plaintiff's claimed injuries or damages. See Paragraph 11(i)

Plaintiff's remaining objections to the Declaration of Mark Needham, MD are OVERRULED.

IV. Legal Standard

Standard for Motion for Summary Judgment – Summary judgment is proper “if all the papers submitted show that there is no triable issue as to any material fact and that the moving party is entitled to judgment as a matter of law.” (Code Civ. Proc., §437c, subd. (c).) The moving party bears the initial burden of production to make a prima facie showing that there are no triable issues of material fact. (Aguilar v. Atlantic Richfield Co. (2001) 25 Cal.4th 826, 850.) A defendant moving for summary judgment must show either (1) that one or more elements of the cause of action cannot be established or (2) that there is a complete defense to that cause of action. (Id at §437c, subd. (p).) A defendant may discharge this burden by furnishing either (1) affirmative evidence of the required facts or (2) discovery responses conceding that the plaintiff lacks evidence to establish an essential element of the plaintiff's case. If a defendant chooses the latter option, he or she must present evidence “and not simply point out that plaintiff does not possess and cannot reasonably obtain needed evidence.” (Aguilar, supra, 25 Cal.4th at 865-66.)

Under the latter approach, a defendant's initial evidentiary showing may ‘consist of the deposition testimony of the plaintiff's witnesses, the plaintiff's factually devoid discovery responses, or admissions by the plaintiff in deposition or in response to requests for admission that he or she has not discovered anything that supports an essential element of the cause of action.’ [Citation.] In other words, a defendant may show the plaintiff does not possess evidence to support an element of the cause of action by means of presenting the plaintiff's factually devoid discovery responses from which an absence of evidence may be reasonably inferred. [Citation.]

(Leyva v. Garcia (2018) 20 Cal.App.5th 1095, 1103 (internal citations omitted).)

Until the moving defendant has discharged its burden of proof, the opposing plaintiff has no burden to come forward with any evidence. Once the moving defendant has discharged its burden as to a particular cause of action, however, the plaintiff may defeat the motion by producing evidence showing that a triable issue of one or more material facts exists as to that cause of action. (See Code Civ. Proc. § 437c(p)(2).) On a motion for summary judgment, the moving party's supporting documents are strictly construed and those of his opponent liberally construed, and doubts as to the propriety of summary judgment should be resolved against granting the motion. (D'Amico v. Board of Medical Examiners (1974) 11 Cal.3d 1, 21.)

The court must grant a motion for summary judgment if, after all admissible evidence is considered, there is no triable issue as to any material fact. (See Code Civ. Proc. § 437c(c).) The court has no discretion to deny a summary judgment motion where the moving party has met their burden and the opposing party has failed to establish any triable issue of material fact exists, and the moving party is entitled to judgment as a matter of law. (Krasley v. Superior Court (1980) 101 Cal.App.4th 59, 70.)

A party may also move for summary adjudication as to one or more causes of action within an action, one or more claims for damages, or one or more issues of duty if the party contends the cause of action has no merit, that there is no merit to a claim for damages, or that one or more defendants did not owe a duty to the plaintiff. (Code Civ. Proc., § 437c, subd. (f)(1).) A motion for summary adjudication may be made by itself or as an alternative to a motion for summary judgment, shall proceed in all procedural respects as a motion for summary judgment, and shall only be granted if it completely disposes of a cause of action, a claim for damages, or an issue of duty. (Id at § 437c, subd. (f)(1)-(2).)

V. Discussion

Application – Rivas moves for summary judgment, or in the alternative summary adjudication. The causes of action asserted against Rivas are: (1) Professional Medical Negligence (Medical Malpractice); (2) Willful Misconduct; (4) Fraudulent Concealment; (5) Lack of Informed Consent; and (8) Negligent Hiring, Training, Supervision, and Retention.

a. Rivas' Immunity From Suit and Liability Pursuant to the PREP Act

i. PREP Act

The Public Readiness and Emergency Preparedness Act, known as the PREP Act, provides that "a covered person shall be immune from suit and liability under Federal and State law with respect to all claims for loss caused by, arising out of, relating to, or resulting from the administration to or the use by an individual of a covered countermeasure." (42 U.S.C. § 247d-6d, subd. (a)(1).) PREP Act immunity applies to any claim "that has a causal relationship with the administration to or use by an individual of a covered countermeasure, including a causal relationship with the design, development, clinical testing or investigation, manufacture, labeling, distribution, formulation, packaging, marketing, promotion, sale, purchase, donation, dispensing, prescribing, administration, licensing, or use of such countermeasure." (42 U.S.C., § 247d-6d, subd. (a)(2).)

The PREP Act is invoked when "the [HHS] Secretary makes a determination that a disease or other health condition or other threat to health constitutes a public health emergency, or that there is a credible risk that the disease, condition, or threat may in the future constitute such an emergency." (42 U.S.C. § 247d-6d, subd. (b)(1).) The Secretary "controls the scope of immunity through the declaration and amendments, within the confines of the PREP Act." (Estate of Maglioli v. Alliance HC Holdings LLC (2021) 16 F.4th 393, 401.) The Secretary's declaration "may specify[] the manufacture, testing, development, distribution, administration, or use of one or more covered countermeasures." (42 U.S.C. § 247d-6d, subd. (b)(1).) The PREP Act created the Covered Countermeasure Process Fund to compensate "eligible individuals for covered injuries directly caused by the administration or use of a covered countermeasure pursuant to such declaration." (42 U.S.C. § 247d-6e, subd. (a).)

The PREP Act provides that "the sole exception to the immunity from suit and liability of covered persons ... shall be for an exclusive Federal cause of action against a covered person for death or serious physical injury proximately caused by willful misconduct ... by such covered person." (42 U.S.C. § 247d-6d, subd. (d)(1).) Such an action "shall be filed and maintained only in the United States District Court for the District of Columbia." (42 U.S.C. § 247d-6d, subd. (e)(1).) The term "willful misconduct" is defined as an act or omission that is taken to achieve a wrongful purpose, knowingly without justification, and in disregard of a known or obvious risk that is so great as to make it highly probable that the harm will outweigh the benefit. (42 U.S.C. § 247d-6d, subd. (c)(1)(A)(i)-(iii).) The "willful misconduct" standard shall be construed as establishing a standard for liability that is more stringent than a standard of negligence in any form or recklessness. (42 U.S.C. § 247d-6d, subd.

(c)(1)(B). An individual may not bring a suit under 42 U.S.C. § 247d-6d, subd. (d)(1) unless the individual has exhausted the remedies available under § 247d-6e, subd. (a), the Covered Countermeasure Process Fund. (42 U.S.C. § 247d-6e, subd. (d)(1).)

ii. COVID-19

On March 10, 2020, the Secretary issued a declaration under the PREP Act "to provide liability immunity for activities related to medical countermeasures against COVID-19." (Rivas RJN, Ex. A; Declaration Under the Public Readiness and Emergency Preparedness Act for Medical Countermeasures Against COVID-19, 85 Fed. Reg. 15,198, 15,198 (Mar. 17, 2020).) The declaration provided immunity for covered persons for the use of covered countermeasures.

Covered countermeasures include "any antiviral, any other drug, any biologic, any diagnostic, any other device, or any vaccine, used to treat, diagnose, cure, prevent, or mitigate COVID-19... or any device used in the administration of any such product." (Id. at 15,202.) When used in respect to the administration of a covered countermeasure, "covered person" includes any person or entity that is a distributor of such countermeasure or a qualified person who administered or dispensed such countermeasure and any employee thereof. (42 U.S.C. § 247d-6d, subd. (i)(2)(B)(ii)-(v).) A "distributor" means a person or entity engaged in the distribution of drugs, biologics, or devices, including retail pharmacies. (42 U.S.C., § 247d-6d, subd. (i)(3).) "A qualified person means a licensed health professional or other individual authorized to prescribe, administer, or dispense Covered Countermeasures under the law of the state in which the Covered Countermeasure was prescribed, administered, or dispensed; or a person within a category of persons identified as qualified in the Secretary's Declaration." (Rivas RJN, Ex. A, 15,199.)

The March 10, 2020 Secretary's Declaration under the PREP Act discusses administration of covered countermeasures as follows:

The PREP Act does not explicitly define the term "administration" but does assign the Secretary the responsibility to provide relevant conditions in the Declaration. In Section IX of the Declaration, the Secretary defines "Administration of a Covered Countermeasure," as follows:

Administration of a Covered Countermeasure means physical provision of the countermeasures to recipients, or activities and decisions directly relating to public and private delivery, distribution, and dispensing of the countermeasures to recipients; management and operation of countermeasure programs; or management and operation of locations for purpose of distributing and dispensing countermeasures.

The definition of "administration" extends only to physical provision of a countermeasure to a recipient, such as vaccination or handing drugs to patients, and to activities related to management and operation of programs and locations for providing countermeasures to recipients, such as decisions and actions involving security and queuing, but only insofar as those activities directly relate to the countermeasure activities. Claims for which Covered Persons are provided immunity under the Act are losses caused by, arising out of, relating to, or resulting from the administration to or use by an individual of a Covered Countermeasure consistent with the terms of a Declaration issued under the Act. Under the definition, these liability claims are precluded if they allege an injury caused by a countermeasure, or if the claims are due to manufacture, delivery, distribution, dispensing, or management and operation of countermeasure programs at distribution and dispensing sites.

Thus, it is the Secretary's interpretation that, when a Declaration is in effect, the Act precludes, for example, liability claims alleging negligence by a manufacturer in creating a vaccine, or negligence by a health care provider in prescribing the wrong dose, absent willful misconduct. Likewise, the Act precludes a liability claim relating to the management and operation of a countermeasure distribution program or site, such as a slip-and-fall injury or vehicle collision by a recipient receiving a countermeasure at a retail store serving as an administration or dispensing location that alleges, for example, lax security or chaotic crowd control. However, a liability claim alleging an injury occurring at the site that was not directly related to the countermeasure activities is not covered, such as a slip and fall with no direct connection to the countermeasure's administration or use. In each case, whether immunity is applicable will depend on the particular facts and circumstances.

(Rivas RJN, Ex. A; Declaration Under the Public Readiness and Emergency Preparedness Act for Medical Countermeasures Against COVID-19, 85 Fed. Reg. 15,198, 15,200 (Mar. 17, 2020).)

iii. Rivas' Burden

Rivas moves for summary judgment on the grounds that he is immune from suit and liability under the PREP Act. To establish immunity, Rivas must establish that he was acting as covered person, engaged in the administration of a covered countermeasure, and that Plaintiff's injury was caused by, arose out of, related to, or resulted from said administration.

It is undisputed that pursuant to the statutory framework set forth, administration and distribution of COVID-19 vaccines are designated as "covered countermeasures" under the PREP Act. (Plaintiff's UMF ("PSUMF") No. 2.) Plaintiff presented to the CVS on April 7, 2021 for the purpose and intent of receiving a COVID-19 vaccine. (PSUMF, No. 10.)

As to Rivas, however, the parties dispute two key issues. First, Plaintiff disputes that Rivas is a "covered person" under the PREP Act. Second, the parties dispute whether the first insertion of a needle during the vaccination process that did not contain or administer any dose of the COVID-19 vaccine and Plaintiff's harm stemming therefrom is causally connected to the administration of a covered countermeasure of administering the vaccine, where Plaintiff alleges the needle contained hazardous substance or was previously used. Defendant argues the injection is expressly covered by the PREP Act, as the first needle was injected and intended for administration of the vaccine, but was by human error not pre-dosed with the vaccination, while Plaintiff argues the first injection "had nothing to do with a Covid 19 vaccine." (UMF, No. 11.)

1. Rivas is a Covered Person Under the PREP Act

Turning to the first disputed fact regarding Rivas' PREP immunity, Rivas argues he is a covered person under the PREP Act because at all relevant times, Rivas was acting as the store manager for CVS in connection with vaccine preparation and administration. (Motion, p. 5:9-12, 7:1-4.)

To reiterate, a "covered person" includes any person or entity that is a distributor, manufacturer, or program planner of such countermeasure, a qualified person who administered or dispensed such countermeasure, or any official, agent, or employee of a covered person or entity described. (42 U.S.C. § 247d-6d, subd. (i)(2)(B)(ii)-(iv); See also Rivas RJN, Ex. A, 15,201, section V.) A "distributor" means a person or entity engaged in the distribution of drugs, biologics, or devices, including retail pharmacies. (42 U.S.C., § 247d-6d, subd. (i)(3).) "A qualified person means a licensed health professional or other individual authorized to prescribe, administer, or dispense Covered Countermeasures under the law of the state in which the Covered Countermeasure was prescribed, administered, or dispensed; or a person within a category of persons identified as qualified in the Secretary's Declaration." (Rivas RJN, Ex. A, 15,199.)

It is undisputed is that Defendants CVS and CVS Health were authorized to administer the COVID-19 vaccine and constitute "covered persons" for the purposes of the PREP Act. (Plaintiff's UMF No. 6.) Rivas argues that he is not a licensed or certified healthcare professional, and therefore, concedes he is not a qualified person for purposes of the PREP Act. (Motion, p. 9:9-25; Rivas Decl., ¶¶ 5-15.) However, Rivas declares that in April 2021, he was employed as the General Manager of the CVS store located at 2006 West Avenue J, Lancaster, CA 93536 where the incident occurred, and that he primarily worked in the front retail portion of the store, overseeing front-store operations and employees who worked in the area. (Rivas Decl., ¶¶ 2-3.)

Rivas also provides the Declaration of Sneha Patel, a District Leader for CVS. (Patel Decl., ¶¶ 1-2.) Patel declares that her responsibilities included overseeing the Palmdale CVS location where Plaintiff received her vaccine and where Rivas and Ifejika were employed in April of 2021. (Patel Decl., ¶ 3.) Patel asserts that prior to April 7, 2021, she was unaware of any complaints regarding Rivas' mishandling of customer complaints or performance as store manager relating to Covid-19 issues, policies, or procedures, and did not receive, review, or become aware of any reports, documentation, or communications to suggest that Mr. Rivas engaged in conduct that would make him unfit for his role as store manager. (Patel Decl., ¶¶ 5-7.)

Rivas provides portions of Plaintiff's deposition testimony from October 23, 2025. Plaintiff testified to the following relevant facts: (1) after the subject incident, Plaintiff returned to the CVS and requested to speak to a manager, and an employee informed her a manager was unavailable and wrote down the name of the manager and provided it to Plaintiff (Ex. 8, p. 27:3-9); (2) Plaintiff subsequently returned to the CVS and requested to speak to a manager, at which point she came into contact with Rivas (Ex. 8, p. 27:10-15); and (3) Rivas, described by Plaintiff as "the manager," brought Plaintiff to an office and placed a call to make a report regarding the incident (Ex. 8, p. 27:21-28:2). Plaintiff again testified on February 27, 2026 to the following relevant facts: (1) Plaintiff is suing the technician, Ifejika, the store manager, Rivas, and the company, CVS (Ex. 12, p. 122:10-13), and (2) at no point prior to, during, or after the incident did Plaintiff know whether Rivas was a medical professional (Ex. 12, p. 122:14-19).

Finally, the Court notes that Plaintiff's SAC defines Rivas as an "employee, manager, supervisor, agent of Defendants CVS HEALTH and CVS PHARMACY, and DOES 3 through 100, and manager, employer and/or supervisor of Defendant ILEKA IFEJIKI." (SAC, ¶ 4.) The SAC states that "Defendant CVS HEALTH...was at all times relevant to the incident herein...the employer of Defendant ANTINO RIVAS." (SAC, ¶ 5.)

The evidence provided by Rivas is sufficient to meet his burden of making a prima facie showing that at all relevant times, Rivas was an employee of CVS. As CVS's status as a covered entity is undisputed, Rivas is therefore a covered person under the PREP Act.

2. Rivas Was Engaged in a Covered Countermeasure

Plaintiff's allegations against Rivas specifically consist of the following: (1) Rivas disregarded and showed no concern for the first needle being injected into Plaintiff's arm (SAC, ¶ 61(u)), and (2) Rivas failed to obtain Plaintiff's consent prior to injection of either needle, disposed of the needles, failed to inform Plaintiff of what had happened or remedy the error, and was unfit and incompetent to perform his duties (SAC, ¶ 88.) Otherwise, the SAC alleges actions against CVS Defendants and Ifejika, all of which are entirely and exclusively related to injections of the first and second syringes. (See SAC generally.)

As a covered person, Rivas is only immune under the PREP Act under certain circumstances. As stated, covered countermeasures include any vaccine or device used to treat, diagnose, cure, prevent, or mitigate Covid-19, or any device used in the administration of such a product. (Rivas RJN, Ex. A, 15,202.) However, the scope of immunity is sweeping. As delineated in the Secretary's Declaration, "[c]laims for which Covered Persons are provided immunity under the Act are losses caused by, arising out of, relating to, or resulting from the administration to or use by an individual of a Covered Countermeasurethese liability claims are precluded if they allege an injury caused by a countermeasure, or if the claims are due to manufacture, delivery, distribution, dispensing, or management and operation of countermeasure programs at distribution and dispensing sites....the Act precludes a liability claim relating to the management and operation of a countermeasure distribution program or site." (Rivas RJN, Ex. A, 15,200.)

Returning to Rivas' Declaration, he states his first encounter with Plaintiff occurred on April 7, 2021, when she returned to the CVS location and expressed concerns over her vaccination encounter. (Rivas Decl., ¶ 17.) Rivas declares he assisted Plaintiff by documenting her complaint and completing an incident report so the matter could be reviewed by CVS Risk Management, and that his role in the incident was limited to documenting the customer complaint and following appropriate internal reporting procedures. (Rivas Decl., ¶ 18.)

As discussed above, Plaintiff testified in her deposition that she returned to the CVS for the sole purpose of speaking with a manager regarding her COVID-19 vaccination appointment, and that the scope of her meeting with Rivas was limited to reporting the interaction and injection of the syringes by Ifejika to an individual in a management capacity. (Ex. 8, p. 27:13-28:2.)

Rivas has sufficiently established that at all relevant times, he was acting in the scope of his employment as a CVS store manager, that his role was limited to reporting Plaintiff's incident complaint related to CVS's administration of the COVID-19 vaccine, and of the few allegations pertaining to Rivas alleged in the SAC, all are specifically related to either Rivas'

operation, management, internal reporting, and handling of Plaintiff's complaints regarding administration of the COVID-19 vaccine, or to his allegedly concealing evidence of the first injection. Accordingly, at all times relevant to this action, Rivas was engaged in conduct related to the management and operation of the specific CVS location for the purpose of distributing and dispensing countermeasures, and was therefore engaged in a covered countermeasure.

3. Plaintiff's Injury Related to Administration of a Covered Countermeasure

Rivas contends that Plaintiff's claims against him arise from or relate to the administration of a covered countermeasure. Rivas submits the Declaration of Defendant Ifejika in support. Ifejika declares that he was employed as a COVID-19 Travel Vaccine Pharmacist for Defendants beginning in January of 2021 to assist in vaccination clinics during the pandemic. (Ifejika Decl., ¶ 2.) He states that during the relevant period in April of 2021, the customary protocol for administering COVID-19 vaccines involved one pharmacist administering vaccines and one nurse assisting by pre-drawing doses from multi-dose vials into single-use, retractable syringes for efficient and safe administration. (Id at ¶ 4.) Ifejika also provides that both of the syringes used were VanishPoint Retractable syringes with an automated retraction technology used for the administration of the COVID-19 vaccine, which are single-use and designed such that once the plunger is depressed after medication delivery, the needle automatically retracts into the barrel, rendering it unusable and eliminating the possibility of reuse or accidental needle sticks. (Id at ¶ 5.) He states that if the plunger is already fully depressed before insertion and the needle has not been retracted, the syringe contains no medication and cannot inject any substance into a patient. (Ibid; Defendants' Ex. 10.)

On April 7, 2021, Ifejika, based on his prior experience at that vaccination location, assumed the nurse on staff had pre-batched and pre-filled the syringes in preparation for the vaccination line, and when Plaintiff arrived for her appointment, he selected what he believed as a pre-filled VanishPoint syringe from the prepared batch, but noticed for the first time upon insertion that the plunger was fully depressed and no liquid was present. (Id at ¶¶ 6-9.) Ifejika states that no air or any other substance could have been delivered

into Plaintiff's arm as the plunger was fully depressed. (Id. at ¶ 10.) He further states that

the syringe could not have previously been used as the needle had not retracted. (Id.)

Ifejika states he then removed the empty syringe, properly discarded it in a Sharps container per standard protocol, cleaned Plaintiff's arm, and retrieved a new, pre-filled VanishPoint syringe containing the COVID-19 vaccine and administered the vaccine to Plaintiff. (Id at ¶ 11.) Ifejika further declares that the single-use retractable VanishPoint syringes used to administer the vaccine eliminate the possibility of cross-contamination or reuse of a needle, and that to a reasonable degree of medical probability, there was no circumstance under which Plaintiff could have been injected with a used needle or needle containing any substance at all, including air. (Id at ¶ 17.)

Rivas also provides the Declaration of Dr. Kimberly Moore, a retained expert and licensed registered pharmacist with extensive experience administering the COVID-19 vaccine in retail pharmacy settings. (Moore Decl., ¶¶ 1, 4; Defendants' Exhibit 11.) Moore declares that based on her experience, in early 2021, COVID-19 vaccines were only available in multi-use vials that required an individual dose to be drawn into a single-use syringe to then be injected into the patient's deltoid muscle. (Moore Decl., ¶¶ 8, (d)-(e).) Moore states that an empty-needle insertion during a high-volume administration setting of a vaccine, such as the administration of the COVID-19 vaccine during the pandemic, is a recognized occurrence, which may occur due to syringe malfunction, mechanical defect, plunger positioning, or human factors, even where appropriate care is exercised, and that empty needle insertion is not a breach of the standard of care, but rather, is a known risk inherent in the vaccination process during the pandemic. (Moore Decl., ¶ 8 (f), (g).) Moore further contends that if a pharmacist reasonably believes an empty-needle insertion occurred and the vaccine dose was not delivered, the appropriate and acceptable standard of care is to administer a replacement dose to ensure the patient receives the intended dose as such action is medically appropriate, reasonable, and consistent with CDC guidance and pharmacy vaccination protocols. (Moore Decl., ¶ 8 (h), (i).) Moore also states that failing to administer a second injection when the pharmacist believes the initial injection did not deliver the vaccine would fall below the standard of care. (Moore Decl., ¶ 8 (j).)

As to the case at hand, Moore states that Plaintiff presented to CVS and consented to receive the COVID-19 vaccine on April 7, 2021, the needles used on Plaintiff were the VanishPoint syringes with automated retraction technology, and that to a reasonable degree of medical probability, on review of the video evidence of Plaintiff receiving the injection, the first injection was empty, and because the needle was not retracted, to a reasonable degree of medical probability, the first injection could not have been a used needle. (Moore Decl., ¶ 8 (k), (l).) Therefore, Moore states that to a reasonable degree of medical probability, the second injection administered contained the COVID-19 vaccine and ensured Plaintiff received the intended vaccination. (Moore Decl., ¶ 8 (n), (o).)

Rivas further submits the Declaration of Mark Richard Needham, M.D., M.B.A., a physician with experience in vaccination procedures and the mechanisms, risk and medical evaluations of needlestick injuries, and adverse events related to the COVID-19 vaccine. (Needham Decl., ¶¶ 1-2, 6.) Needham states that after review of the records and video evidence of the vaccination encounter, the plunger of the first syringe used to administer the vaccine was fully depressed at the time of insertion, which from a medical and mechanical standpoint, indicates the syringe was empty, no medication or substance was injected, and that an empty, single-use syringe cannot transmit medication, infectious material, or contaminants. (Needham Decl., ¶ 11 (a)-(c).)

Rivas submits transcripts of Plaintiff's depositions. Plaintiff testified to the following relevant facts:

Plaintiff presented to the CVS Premises for the purpose of receiving the COVID-19 vaccine after scheduling the vaccination appointment in advance. (Ex. 8, p. 20:25, 22:11-12.) Plaintiff scheduled an appointment to receive the COVID-19 vaccine and received confirmation from CVS of the appointment. (Ex. 8, p. 20:20-25.) Plaintiff arrived for the vaccine at approximately 8:50 a.m., 10 minutes before her 9:00 a.m. appointment, and waited approximately 15 minutes. (Ex. 8, p. 22:9-21.) Plaintiff was then escorted to the back of the pharmacy to the cubicles where the person in charge of doing the vaccine was located. (Ex. 8, p. 22:22-23:4.) Plaintiff asked another customer in line for vaccination to take a photograph of her receiving the injection, this individual asked Ifejika at the time of injection "two shots?", and Plaintiff felt two needles inserted in her arm. (Ex. 8, p. 25:1-3, 30:5-25, 31:7-11.) Plaintiff did not see either the first or second shot being prepared, sealed, or prefilled, did not see whether the syringes appeared new or used, and did not see anything regarding the source or disposal of either needle. (Ex. 8, p. 31:7-33:21.) Plaintiff did not see what Ifejika did with the first needle after injection, disposal or otherwise. (Ex. 8, p. 38:2-16.) The second shot Plaintiff received was a COVID-19 vaccine, and Plaintiff states the reason she was there was to receive the COVID-19 vaccine. (Ex. 8, p. 40:23-12.) At her later deposition, Plaintiff stated she does not know whether the second syringe contained the COVID-19 vaccine. (Ex 12, p. 118:17-119:7.) Attached to Exhibit 8 and 12 are photographs of Plaintiff receiving both injections.

There is little published case law on PREP immunity. However, as to summary judgment motions specifically, non-binding, but persuasive decisions from various jurisdictions have established the need for the defendant to evidence a causal connection between a plaintiff's loss and a covered countermeasure. (See *Austbo v. Greenbriar Nursing Home No. Two, Inc.* (2025) OK 85, 20, 23; *Wilhelms v. ProMedica Health Sys., Inc.* 2023- Ohio 143, 205 N.E.3d 1159, 1161 (Ohio Ct. App. 2023); *Ashley v. Anonymous Physician 1*, 245 N.E.3d 658, 2024 WL 4142508, at *1 (Ind. Ct. App. 2024).)

Here, the Court finds that Rivas' evidentiary showing is sufficient to satisfy his burden of showing that the injection at issue was directly related to CVS's administration of countermeasure activities, that any injury Plaintiff allegedly suffered as a result of the first injection was caused in the process and for the purpose of administration of the COVID-19 vaccine, and that her allegations against Rivas are exclusively related to his capacity as manager of the approved countermeasure administration location. The covered countermeasure at issue is the COVID-19 vaccine. Plaintiff made an appointment to get the vaccine; she showed up for her appointment; she met with Ifejika to get the vaccine; and she in fact received a vaccination shot from Ifejika (as even she admitted at her first deposition). The PREP Act does not require that a covered countermeasure be successfully delivered by every needle insertion; it immunizes any loss "arising out of, relating to, or resulting from the administration" of the countermeasure, and the management and operation of a location administering countermeasures. (42 U.S.C. § 247d-6d, subd. (a)(1).) Ifejika's selection and use of a syringe he believed contained the vaccine, drawn from the batch designated for its administration, is an activity and decision directly relating to the delivery and dispensing of that vaccine. Plaintiff then met with Rivas after attempting numerous times to meet with a manager regarding the administration of the vaccine and reported her complaints. Any acts taken by Rivas were taken in his role as a manager in responding to COVID-19 vaccine-specific customer complaints, making hiring decisions for the staffing specifically of the vaccine administration program located and operated at the Lancaster CVS, general management and operation of the premises, decisions and policies regarding patient consent, and policies and practices of vaccine administration and disposal of syringes. All of these activities directly relating to the management and operation of an authorized countermeasure location.

The plain language of the PREP Act is broad and sweeping in scope. As discussed, administration of a covered countermeasure includes not only the physical provision of a countermeasure to a recipient, such as the direct injection of the vaccine itself, but also includes activities related to management and operation of locations for providing countermeasures, including decisions and actions made, so long as they directly relate to the countermeasure activities. (Rivas' RJN, Ex. A, 15,200.)

In *Maney v. Brown* (2024) 91 F.4th 1296, the Ninth Circuit Court of Appeals affirmed that PREP Act immunity is not limited to the actual injection of the vaccine itself. (Maney, *supra*, 91 F.4th at 1301.) The Court reasoned:

The phrasing "administration to...an individual of a covered countermeasure" could refer only to the act of physically giving a countermeasure to a particular person—for example, injecting someone with a vaccine shot. However, in § 247d-6d(a)(2)(B), the Act provides that the scope of immunity under paragraph (1) includes various activities with a causal relationship to the administration of a countermeasure beyond injecting someone with a vaccine. Most significantly, subsection (a)(2)(B) lists several terms, including "administration," without reference to "an individual." This is consistent with the expansive causal relationship the subsection provides; for example, the "design, development," "manufacture," and "distribution" of a vaccine are multiple links removed in the chain of events from the ultimate injecting of an individual with a vaccine. By referring to "administration . . . of [a covered] countermeasure," in the context of a list that expands the conduct within the Act's scope of immunity, and without requiring a direct link to an individual, subsection (a)(2)(B) broadens the scope of immunity to administrative activities other than the physical act of directly injecting a particular person with a vaccine.

(*Ibid* at 1300-01 (emphasis added); See also *Hampton v. California* (9th Cir. 2023) 83 F.4th

754, 764 ["At the very least, then, for PREP Act immunity to apply, the underlying use or

administration of a covered countermeasure must have played some role in bringing about

or contributing to the plaintiff's injury. It is not enough that some countermeasure's use

could be described as relating to the events underpinning the claim in some broad sense."].)

The Secretary's interpretation of "administration of a protected countermeasure" for COVID-19 precludes liability even for dangerous conditions on a property, immunizing covered persons for incidents such as vehicle collisions and premises liability claims, such as slip-and-fall incidents, if there is a direct connection to the countermeasure's administration, such as where an individual suffers such harm while being on the premises for a vaccine and the conditions were created by the operation of a countermeasure program. (Defendants' RJN, Ex. A, 15,200.)

Here, Rivas has submitted competent evidence that the first injection was an activity and decision directly relating to the delivery, distribution, and dispensing of the countermeasures to Plaintiff. Therefore, Plaintiff's allegations against Rivas as the manager of the location directly relate to management and operation of CVS's countermeasures. First, Ifejika's Declaration establishes that the first injection was given in error in the process of administering the COVID-19 vaccine based on his assumption, in accordance with standard vaccination practice at the location, that the syringe had been pre-filled. Second, Dr. Moore declares that empty shots which have not been pre-filled were a recognized occurrence in administration of the COVID-19 vaccines in large-scale vaccination centers at the time Plaintiff received the injections. Third, Plaintiff's deposition testimony corroborates a causal link between the first injection and Defendants' administration of the vaccine as a licensed retail pharmacy. The sole purpose for her presence was to receive the vaccination as advertised by the pharmacy. Fourth, the circumstances set forth in the SAC and Rivas' evidence support this finding. Both injections were given by Ifejika, who is a COVID-19 Travel Vaccine Pharmacist, whose role at CVS was to assist and administer the COVID-19 vaccines. Both injections were given successively in the same seat within the pharmacy designated for vaccine administration and in close temporal proximity, almost in immediate succession. The first shot was given during Plaintiff's scheduled appointment to receive the vaccine and was given with the intent and belief that it contained the vaccine. The same type of syringe was

used for both injections, which was the standard syringe used at the time for COVID-19 vaccine administration, and Ifejika declares the first syringe came from a batch of what he believed were pre-filled COVID-19 vaccine syringes in accordance with the standard administration practices of that specific CVS location. Finally, the SAC, Rivas' Declaration, and Plaintiff's testimony confirm that her allegations against Rivas, including that he disregarded her reported concerns, concealed evidence, failed to obtain Plaintiff's consent, and was negligent in permitting Ifejika to remain employed by CVS all relate directly to the administration efforts at that particular CVS location, Ifejika's injection of two syringes, and Rivas' responsibility in managing administration efforts at the location.

The purpose of the PREP Act is to immunize and shield protected persons from negligence, improper administration, improper management and operation of a countermeasure location, and other tort liability in connection with administration of the COVID-19 vaccine. Rivas' evidence has established that the first injection was an empty, unused, single-use syringe, that Ifejika mistakenly believed the syringe was pre-filled with the vaccine, inserted the needle without realizing the syringe did not contain the vaccine, and promptly thereafter disposed of the syringe and injected a second syringe containing the intended dose. Whether these facts give rise to negligence in the management or operation of the vaccination program premises or administration of the program generally is not relevant, as the PREP Act was expressly designed to preclude actions based thereon.

Therefore, Rivas has presented sufficient evidence to show that the first injection was an act in relation to the administration of a covered countermeasure and that Rivas' alleged conduct is directly related to the management and operation of a countermeasure program. Rivas has established the requisite causal relationship between Plaintiff's allegations against him and his protected conduct in managing a countermeasure program location. The burden shifts to Plaintiff to establish a triable question of fact exists as to whether the injury has a causal connection to the administration of the vaccine.

iv. Plaintiff's Burden

Plaintiff argues Rivas is not a covered person under the PREP Act because he does not fit the definition of a "qualified person" under the PREP Act. (Opp., p. 16:23-17:6.) However, Rivas need not be a "qualified" person to be a "covered" person. A "covered person" includes both (1) a qualified person who administered or dispensed such countermeasure, and (2) any official, agent, or employee of a covered person or entity described, including distributors such as CVS. (42 U.S.C. § 247d-6d, subd. (i)(2)(B)(ii)-(iv); See also Rivas RJN, Ex. A, 15,201, section V.) Plaintiff's SAC concedes that Rivas was an employee and manager of CVS at the time of the incident. Plaintiff relies on Rivas' status as employee and manager throughout her responses to Rivas' statement of undisputed material facts. (See, e.g., PSUMF, Nos. 16, 19-23.) Plaintiff's deposition testimony and filings refer to Rivas exclusively as a manager or employee of CVS. Plaintiff argues Rivas was employed by CVS at all relevant times. (See PSUMF, p. 20, No. 3.) Plaintiff's present motion also argues that Rivas acted in the capacity of an ostensible agent, which if true, would also render Rivas a covered person under the PREP Act as an agent of either CVS or Ifejika, both of whom are "covered persons."

Plaintiff next argues that application of the PREP Act to these circumstances is too attenuated, as "administrative activities" as described by the Maney Court, discussed supra, cannot reasonably reach a blank injection of a syringe not containing the vaccine itself. (Opp., p. 12:12-26.) Plaintiff argues that the phrase "related to" must be viewed with a targeted meaning and based on a clear causal relationship establishing that the use or administration of a covered countermeasure played some role in bringing about or contributing to Plaintiff's injury, not merely that the countermeasure's use could be described as relating to the events underpinning the claim in a broad sense. (Opp., p. 13:2-12.) Plaintiff avers that if the Court were to find that Rivas is entitled to PREP Act immunity merely because Plaintiff eventually received the vaccine after being injected with an empty syringe, the holding would be akin to ruling that anything that could have caused Plaintiff injury the day of her COVID-19 injection therefore relates to the administration of a covered countermeasure. (Opp., p. 13:13-22.)

The problem with Plaintiff's argument lies in her definition of "administration." As discussed above, "administration" encompasses actions, decisions, operations, and general management in relation to covered countermeasures. Plaintiff's premise, that because the first syringe was empty no covered countermeasure was involved, misapprehends the statute. The covered countermeasure is the vaccine Plaintiff came to receive, not the contents of any specific syringe. The immunity reaches any loss relating to or arising out of its administration. That an individual dose went undelivered no more removes the pharmacist's conduct from the administration of the vaccine than would a mismeasured or wrong dose; indeed, the Secretary's Declaration expressly identifies negligence by a health care provider in prescribing the wrong dose as conduct for

which the Act precludes liability. (Defendants' RJN, Ex. A; 85 Fed. Reg. at 15,200.)

In Plaintiff's response to Rivas' Statement of Undisputed Material Facts, Plaintiff does not expressly dispute that "[a]ll of Plaintiff's alleged injuries arise from, relate to, or are connected with the administration and use of a COVID-19 vaccine during the COVID-19 public health emergency," but merely disputes the appropriate legal standard. (See PSUMF, No. 22.)

In opposition to the motion, Plaintiff has provided no evidence refuting Rivas' evidence that the syringe and needle used to administer the first shot was a VantagePoint syringe, that the first syringe was taken from a batch of syringes specifically designated for administration of the COVID-19 vaccine, or that any other injections or vaccinations were being given, to anyone, in the designated location on the designated date. Plaintiff has also submitted no evidence establishing that the first injection was not given in the process of COVID-19 vaccine administration or that there was no causal connection between Ifejika's administration of the vaccine and the injection. The Declaration of Ruwanthi Campano, MD does not put forth any evidence that refutes Defendant's showing that they were engaged in a covered countermeasure. Campano's Declaration actually appears to substantiate this characterization. (Campano Decl., ¶ 9 ["This has nothing to do with the injection of a covid-19 vaccine. This has to do with the injection technique and the lack of informed consent and duty to relate an unforeseen occurrence in the administration of a vaccine by this pharmacist on April 7, 2021, that is what is being called into dispute."].) Dr. Campano's Declaration is silent as to Rivas. Further, Dr. Campano's opinions on the alleged negligence of Ifejika in administering the two injections (id.) are largely immaterial. The point of Rivas' PREP argument is not that he is blameless of negligence, but that the PREP Act immunizes his actions.

Similarly, the Declaration of John Peterson PharmD speaks to Ifejika's conduct falling below the standard of care, but is silent as to any evidence or facts bearing on PREP immunity. Again, Peterson also appears to characterize the first injection as occurring in the process of "vaccine administration." (Peterson Decl., ¶ 5 (a) ["The pictures of the vaccine administration on April 7, 2021 show the pharmacist injecting the plaintiff with what appears to be an empty syringe and the plaintiff begins to bleed. The pharmacist then immediately wipes the plaintiff's arm with a paper towel and immediately injects the patient with another syringe."].) Peterson makes no mention of Rivas' conduct.

Plaintiff argues that this lawsuit "is about injecting Plaintiff SALAZAR with two needles, the first of which was clearly not part any Covid 19 vaccine administration." (See, e.g., PSUMF, p. 131, No. 17.) However, Plaintiff simultaneously characterizes the first injection as being given in the administration of the vaccine by a pharmacist. (See PSUMF, Nos. 15-19 ["This has nothing to do with the injection of a covid-19 vaccine. This has to do with the injection technique and the lack of informed consent and duty to relate an unforeseen occurrence in the administration of a vaccine by this pharmacist on April 7, 2021, that is what is being called into dispute."]; PSUMF p. 20, No. 3 ["The pictures of the vaccine administration on April 7, 2021 show the pharmacist injecting the plaintiff with what appears to be an empty syringe and the plaintiff begins to bleed."]; PSUMF, p. 28, No. 7 [in response to Defendants' statement of fact that "Defendants [] did not intend to harm Plaintiff in the administration of her COVID-19 vaccine," Plaintiff does not dispute that the first injection was given in the administration of her vaccine, but disputes on the grounds that "IFEJKA never looked at the first syringe before injecting it."].)

As to Rivas specifically, Plaintiff's motion is silent. The SAC does not assert any causes of action against Rivas for any conduct separate and apart from his role as manager and his handling of the first syringe and the incident after it was reported. Plaintiff alleges that Rivas did not properly handle a complaint related to administration of a protected countermeasure as the manager of the location, that he concealed evidence of the first syringe, that he failed to obtain Plaintiff's consent for the first or second injection, and that he failed to provide an incident report following Plaintiff's complaint. Plaintiff opposes Rivas' separate statement in part by arguing that Rivas engaged in further wrongdoing by permitting Ifejika to remain employed at the Lancaster CVS location as a COVID-19 travel nurse following her reporting of the incident. (See, e.g., PSUMF, No. 25.) This assertion, as with the others against Rivas, is directly related to Rivas' management and operation of a designated CVS location for administering covered countermeasures, arises entirely from complaints related to vaccine administration, and falls squarely within immunized conduct under the PREP Act.

Plaintiff provides excerpts from Rivas' deposition testimony, in which he states every employee of CVS is expected to report injuries and incidents, that he emailed his District Leader, Patel, to report the incident, and that his personal custom and practice as store manager is to report incidents occurring in his CVS location by the end of the day it was reported. (Plaintiff's Ex. J, p. 133:14-24, 147:16-148:16.) This testimony is irrelevant to the issue of PREP immunity, and no other evidence regarding Rivas' conduct is presented.

Finally, as to Plaintiff's argument that her claims arise from non-administration of the vaccine, it is true that many Courts have held that in actions arising from failure to administer covered countermeasures do not confer PREP immunity on defendants. These cases are factually distinguishable. The cases specifically deal with failure to implement any protective measures at all. In *Adler*, the New York County Supreme Court reiterated that where a plaintiff's claims "pertain only to the defendant's failures to act, [] such allegations do not amount to the administration of countermeasures," but PREP is triggered where the plaintiff brings suit for improper administration of countermeasures. (*Adler v. Dewitt Rehab. & Nursing Ctr.*, 2026 N.Y. Misc. LEXIS 5116, *28.)

In *Eaton v. Big Blue Healthcare* (2020) 480 F.Supp.3d 1184, 1195, the plaintiff brought a wrongful death action after his mother died of COVID-19, contracted at a rehabilitation facility. Plaintiff alleged that the facility was negligent in not taking measures to protect his mother from COVID-19. The facility removed the case to federal court, claiming that the PREP Act applied to its conduct. The district court rejected the argument as the complaint alleged defendant had not so much injured the decedent in the course of administration of countermeasures but had failed to protect decedent from COVID-19. (*Id.* at 1194.) "The PREP Act still requires a causal connection between the injury and the

use or administration of covered countermeasures, and that link is not present under

Defendants' interpretation." (*Ibid.*) Here, unlike the decedent in *Eaton*, Plaintiff was a

recipient of countermeasures under the PREP Act, and suffered her alleged injury – the

first injection – as part of that administration.

In *Dupervil v. Alliance Health Operations, LCC* (E.D.N.Y. 2021) 516 F.Supp.3d 238, 256, the Court determined the PREP Act did not apply because (1) failing to administer countermeasures to protect against COVID-19 does not constitute administration of countermeasures for purposes of the Act, and (2) the Court rejected defendant's argument that a facility is per se covered for all claims arising from the facility merely because it was using covered countermeasures somewhere in the facility, where there was no showing of causation and no allegation that the harm suffered was the result of the administration or use of countermeasures on or by the injured party.

In *Coleman v. Sharp Memorial Hosp.*, No. 37-2023 00033307-CU-PO-CTL, 2024 Cal. Super. LEXIS 10893 (Cal. Sup. Ct. March 29, 2024), the Court overruled Defendant's demurrer regarding PREP immunity where the Plaintiff's complaint may have been based on administration of a covered countermeasure, but also, could be read to be based on fraudulent concealment of life-saving treatments separate and apart from any COVID-19 countermeasure. Because multiple interpretations of the complaint's allegations were possible, the Court could not determine as a matter of law whether PREP immunity applied at the pleadings stage. (*Ibid.*)

In *Hampton v. California* (2023) 83 F.4th 754, 763, the Court found that defendants were not entitled to PREP immunity where the plaintiff alleged that the defendants' failure to administer COVID tests to inmates in the days prior to their transfer to another prison was the cause of death of a non-transferred inmate at the receiving prison. The Court rested its ruling on two grounds: (1) allegations regarding failure to test are not covered under the PREP Act, as the Act only reaches injury in the active administration of a countermeasure, and (2) to the extent that the complaint alleged the tests used before transfer were so outdated as to be ineffective, the complaint also alleged that the test results came back after the inmates who tested positive had already been transferred and housed in the same unit as the decedent, the defendants failed to establish a causal relationship between the test results and their decision to transfer the inmates, and therefore administration of COVID tests did not ultimately cause the decedent's death. (*Id.* at 763-64.)

Here, Plaintiff is not bringing suit for Rivas' failure to take any response in reaction to the danger posed by COVID--19. Rather, based on the unrefuted evidence provided by Defendants, Plaintiff brings suit for Defendants' improper injection in the course of administration of a protected countermeasure, Rivas' subsequent concealment of evidence, Rivas' retention of

Ifejika as an employee, and Rivas' negligent management of the Lancaster CVS location in administration of a protected countermeasure. Defendants, who are protected persons under PREP, were actively engaging in distribution, administration, and management of the COVID-19 vaccine and location of administration of a protected countermeasure. Plaintiff was only on site to receive the vaccine. Plaintiff was seated in the designated COVID-19 vaccination area. Plaintiff received both the first and second shots while seated to receive a scheduled COVID-19 vaccine. Plaintiff received both shots from Ifejika, who was employed as a COVID-19 Travel Vaccine Pharmacist, whose role was to administer COVID-19 vaccines. Plaintiff lodged a complaint with Rivas, manager of the location, solely related to this encounter. As discussed, this evidence is sufficient to show that a covered person, engaged in administration of covered countermeasures, caused the harm alleged. The Court cannot find based on the evidence presented that Rivas' conduct related to the first injection was unrelated to the management and operation of a location engaged in the administration of a covered countermeasure.

Accordingly, Defendant Rivas, as an employee of CVS, is entitled to PREP Act immunity, and Rivas' motion for summary judgment is therefore GRANTED.

Because the sole exception to PREP Act immunity is an exclusive federal cause of action that may be pursued only after the plaintiff exhausts the Countermeasures Injury Compensation Program and then only in the United States District Court for the District of Columbia (42 U.S.C. §§ 247d-6d, subds. (d)(1), (e)(1), 247d-6e, subd. (d)(1)), this Court lacks jurisdiction to adjudicate a claim of willful misconduct. Plaintiff's second cause of action for willful misconduct therefore cannot proceed in this forum, and the Court does not reach whether Plaintiff could otherwise satisfy that heightened standard.

VI. Conclusion

Defendant Antino Rivas's Motion for Summary Judgment is GRANTED.